BASELINE → TREATMENT → FOLLOW-UP

Required safety checks and your individual disease-assessment plan

The team supplies actual dates and resolves any source ambiguity. A reference period does not replace a prescription, a local monitoring plan or an individual treatment decision.

Check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety	Full blood count, kidney/liver tests, glucose and thyroid tests; hepatitis B surface antigen, hepatitis B core antibody and hepatitis C testing. Review autoimmune conditions, organ transplantation, previous immune reactions and medicines.	Full blood count, kidney/liver tests and glucose each cycle; thyroid-stimulating hormone (TSH) every three to six weeks. Review new symptoms at every visit and arrange additional tests when indicated. Response assessment and scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Immune effects can begin or persist after treatment stops. Keep the agreed follow-up, steroid or hormone-replacement instructions and alert card, and tell clinicians about previous cemiplimab. The next scan, skin/lesion review and any further treatment are individually planned.
Cancer, skin and daily function	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up includes examination of the treated area, the rest of the skin and relevant lymph nodes. Imaging and specialist visits depend on the extent of disease, risk features, symptoms and treatment response. Keep a clear wound and skin-surveillance plan alongside oncology visits.
Medicines and continuing precautions	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00812 v3a: current regimen source · NCCN Guidelines for Patients: Squamous Cell Skin Cancer, 2023 - supplied education reference

THE WHOLE TREATMENT JOURNEY

Cemiplimab for advanced cutaneous squamous cell cancer: treatment and checks

NCCP 00812b: metastatic or locally advanced cutaneous squamous cell carcinoma when curative surgery or curative radiotherapy is not appropriate. This guide covers skin SCC, not the separate cervical-cancer indication.

Cemiplimab is given by IV infusion on Day 1 every 21 days. The usual source dose is 350 mg over 30 minutes; the prescription and visit checks confirm your treatment. There is no fixed cycle total in this regimen: treatment continues while the team considers it beneficial and tolerable.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before starting	During treatment	Between visits	Response / follow-up	After a hold or stop
Bloods and organ safety NCCP 00812	Full blood count, kidney/liver tests, glucose and thyroid tests; hepatitis B surface antigen, hepatitis B core antibody and hepatitis C testing. Review autoimmune conditions, organ transplantation, previous immune reactions and medicines.	Full blood count, kidney/liver tests and glucose each cycle; thyroid-stimulating hormone (TSH) every three to six weeks. Review new symptoms at every visit and arrange additional tests when indicated. Response assessment and scans follow the individual plan.	Contact the team for new symptoms; extra tests may be needed between scheduled checks.	Safety bloods do not by themselves show whether cancer is responding.	Immune effects can begin or persist after treatment stops. Keep the agreed follow-up, steroid or hormone-replacement instructions and alert card, and tell clinicians about previous cemiplimab. The next scan, skin/lesion review and any further treatment are individually planned.
Cancer, skin and daily function NCCP 00812	Record diagnosis, stage, current disease sites and the baseline assessment.	Keep the source-specific skin and clinical reviews and the individually agreed response assessments.	Record new lesions, lumps, wound changes, pain and effects on usual activities.	Write the next examination or scan date and the responsible team; no universal scan or tumour-marker interval is assumed.	Follow-up includes examination of the treated area, the rest of the skin and relevant lymph nodes. Imaging and specialist visits depend on the extent of disease, risk features, symptoms and treatment response. Keep a clear wound and skin-surveillance plan alongside oncology visits.
Medicines and continuing precautions NCCP 00812	Review all medicines and the pregnancy/fertility plan.	Review treatment taken or received, holds, adverse effects and supportive medicines.	Keep the actual urgent contacts and do not restart a held medicine yourself.	Bring the medicine list and treatment summary to other services.	Use effective contraception during cemiplimab treatment and for at least four months after the last dose. Discuss fertility before treatment, and follow the medicine-specific breastfeeding restrictions. Report a possible pregnancy promptly.

What each check is for

1

Cemiplimab treatment

Every three weeks

Cemiplimab helps immune cells act against advanced cutaneous squamous cell cancer when curative surgery or radiotherapy is not suitable. The team reviews cancer control, symptoms, side effects and your priorities to decide whether to continue.

Ask / act: Confirm the current treatment phase, prescription, tests and support medicines.

Sources: NCCP 00812

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Sources checked 2026-11-06 · Draft source reconciliation; independent clinical, pharmacy and patient/PPI review pending · SACTCheck v0.84.40